

WHITMORE, Harold James

DOB: March 8, 1945 | Age: 71 yrs | Sex: Male | MRN: SMC-04471932

Encounter Date: July 7, 2016 | Provider: Raymond Foster, MD — Cardiology | Doc ID: SCA-2016-0707-FU

Cardiology Follow-up

Atrial Fibrillation Management & Echocardiogram Review

Subjective

Two weeks after ED visit for new AF. Palpitations resolved on rate control. No bleeding on apixaban. NYHA Class I-II; mild exertional dyspnea. Adherent to medications.

Vital Signs

BP (mmHg)	HR (bpm)	RR	Temp	SpO2	Weight	BMI
126/78	72 irreg	15	36.6 C	97% RA	94.0 kg	30.6

Echocardiogram (07/01/2016) — Results

LVEF 50% (low-normal). Mild concentric LVH. Left atrium mildly dilated (4.4 cm). Grade I-II diastolic dysfunction. Mild mitral and tricuspid regurgitation. No significant valvular stenosis. Estimated PASP 32 mmHg.

Assessment

- Atrial fibrillation, persistent — adequately rate-controlled; anticoagulated (apixaban) given CHA2DS2-VASc 4.
- Decision: rate-control strategy preferred over rhythm control given age/comorbidity and minimal symptoms.
- CAD s/p DES — stable. Given AF + remote stent, discontinue aspirin (apixaban monotherapy) to reduce bleeding risk.
- CKD stage 3a — will affect future dosing.
- Borderline LV function — monitor.

Plan

- Continue apixaban 5 mg BID; STOP aspirin (anticoagulant monotherapy now that >12 months from PCI).
- Continue metoprolol succinate 100 mg daily for rate control; target resting HR <100.
- Continue atorvastatin, lisinopril.
- Repeat echo in 12 months; cardiology follow-up 6 months.
- Coordinate CKD monitoring with PCP/nephrology.

Updated Cardiac Medications

Medication	Dose	Route	Frequency	Indication
Apixaban	5 mg	PO	Twice daily	AF anticoagulation
Metoprolol succinate	100 mg	PO	Once daily	Rate control / CAD
Atorvastatin	80 mg	PO	Nightly	Secondary prevention
Lisinopril	20 mg	PO	Once daily	HTN / renoprotection
Aspirin	DISCONTINUE D	—	—	Stopped — bleeding risk on OAC